

Adoption of best value denosumab 60mg biosimilar(s) in primary care.

1. Scheme Outline

The purpose of this scheme is to support better value prescribing of denosumab 60mg injection for prevention of osteoporotic fragility fractures in postmenopausal women, in line with NICE guidance¹.

Practices are asked to identify patients prescribed either Prolia or generic denosumab 60mg and switch to a preferred brand in line with formulary recommendations (see Table 1).

This scheme will run from 1st July to 31st December 2026.

2. Background

2.1 National Outlook

A biosimilar medicine (known as a 'biosimilar') contains a version of an active substance of an approved biological medicinal product, known as the reference product. Biosimilar medicines offer the same clinical effectiveness and safety as the reference biologic medicines, but usually at a substantially lower cost^{2,3}.

MHRA has updated the guidance on interchangeability of biosimilar products, stating that *"once authorised, a biosimilar product is considered to be interchangeable with their reference product, which means a prescriber can choose the biosimilar over the reference product (or vice versa) and expect to achieve the same therapeutic effect. A biosimilar product is also considered to be interchangeable with another biosimilar to the same reference product"*⁴.

Denosumab 60mg injection is a monoclonal antibody recommended by NICE for use in prevention of osteoporotic fragility fractures in postmenopausal women¹. Since its launch, it has only been available as the branded product Prolia. The patent of Prolia expired in November 2025. Several lower cost denosumab biosimilars have now entered the market.

In 2025, NHS Devon spent £429,609 on denosumab 60mg injections in primary care. Adoption of these biosimilars could deliver significant financial savings with minimal impact on clinical outcomes^{2,3}. To support consistent use of best-value medicine, three biosimilars have been added to the Devon Formulary and identified as the preferred brands for local switching initiatives^{5,6}.

Meanwhile, primary care prescribing data shows that over 30% of denosumab 60mg prescriptions in Devon were prescribed generically. MHRA guidance states that all biological medicines must be prescribed by brand name to support effective pharmacovigilance⁴. Ensuring appropriate adoption of denosumab biosimilars will therefore support both cost savings and improve medicines safety in practice.

2.2 Patent expiry

Following the patent expiry of Prolia, the market is transitioning to biosimilars with several brands launching in the UK market.

Of the biosimilar products now available, Stoboclo (Celltrion) offers the lowest acquisition cost followed by Osvyrti (Accord), whilst Zadenvi (Zentiva) is the biosimilar of choice for secondary care in the Southwest region following a tender process led by NHS England (Table 1). The Devon Formulary has been updated to include these three as the preferred denosumab 60mg product(s) for Devon. A comparison of the listed biosimilars with Prolia is provided in Appendix 3.

Table 1 - Comparison of May 2026 drug tariff price for Prolia vs acquisition cost for denosumab biosimilars available

Denosumab 60mg product (brand name)	Acquisition cost
Prolia	£183.00
Stoboclo	£54.00
Osvyrti	£67.76
Zadenvi	£109.80

2.3 Dosing information and monitoring requirement

In undertaking this review and change activity, the practice acknowledges that it accepts clinical governance responsibility for completing this work.

The standard adult dose for prevention of osteoporotic fragility fractures in postmenopausal women, in line with NICE TA204¹ is 60mg every 6 months, by subcutaneous injection into the thigh, abdomen or upper arm.

Please note – the prescription must be written as the brand for the practice to receive remuneration.

Practices should ensure that relevant blood results are current before amending prescribed medication. The onus is on the practice to decide if existing results can be used for each patient, considering the patients' individual circumstances, and the required frequency of their blood tests. In some instances, new blood tests may be required.

2.4 Inclusion and exclusion criteria:

Inclusion criteria:

- Prescribing of denosumab 60mg in primary care for prevention of osteoporotic fragility fractures in postmenopausal women

Exclusion criteria:

- Prescribing of denosumab in secondary care
- Prescribing of denosumab 60mg for non-formulary indications
- Prescribing of denosumab 120mg in primary care

3. Summary of Scheme

This scheme will run from 1st July to 31st December 2026. This scheme is open to all GP practices within NHS Devon ICB and is funded at a maximum value of **£25 per patient**.

Practices are asked to review patients prescribed Prolia or generic denosumab 60mg injection. It is the responsibility of the practice to determine how this may be best achieved from a clinical and operational governance perspective.

Eligible patients should then be informed of, and switched to, a preferred denosumab 60 mg biosimilar product where clinically appropriate. Practices will receive remuneration when Prolia or generic denosumab 60 mg injection is prescribed as a preferred brand in accordance with formulary guidance.

3.1 Scheme Activity

The actions for practices are as follows:

Action 1 – Clinical System Searches

Import and run clinical system searches provided by the medicines optimisation team, to identify patients prescribed Prolia or generic denosumab 60mg injection.

Action 2 – Patient Reviews

All patients identified through clinical system searches should be reviewed, and where clinically appropriate, patients should be informed and switched from Prolia or generic denosumab 60mg injection to a preferred brand in line with formulary recommendations.

Action 3 – Data Submission

Complete the [Microsoft form](#) detailing the activity undertaken for patients identified as prescribed a Prolia or generic denosumab 60mg injection within the practice. The completed form will need to be submitted for payment to be claimed, as detailed in section 5 (payment).

4. Eligibility

All practices within Devon are invited to engage with this scheme where there are patients prescribed Prolia or generic denosumab 60mg injection.

Practices do not need to sign up for the scheme and remuneration will be based on submission of evidence and subsequent prescribing data as outlined in section 5 (Payment).

5. Payment

5.1 Standard payments

Practices will receive remuneration based on the number of reviews conducted and the changes to prescribing made:

- a. Practices will receive **£10** per review for patients prescribed Prolia or generic denosumab 60mg injection.
- b. Practices will receive an additional **£15** per patient where a change is made to a preferred brand in line with formulary recommendations.

Where practices make a change from Prolia or generic denosumab 60mg to a preferred brand in line with formulary recommendations, they may claim payment a. and b. The maximum amount available for each patient identified is **£25**. There will be instances where a change is not possible for clinical reasons, in these cases a review payment only may be claimed.

Practices will need to complete clinical activity by 31st December 2026 to be eligible for remuneration as outlined above. Practices are required to submit the completed [Microsoft form](#), detailing the number of reviews and prescribing changes made for the standard payment by 31st January 2027.

The medicines optimisation team will confirm the standard payment amount and invite practices to invoice by 26th February 2027. Invoicing details are included in Appendix 1.

5.2 Secondary care initiations

Where, within the dates of the scheme, patients are commenced on denosumab 60mg injections by secondary care, where possible practices should amend patients' therapy to a preferred brand in line with formulary recommendations. Practices will be remunerated at the standard payment rate providing that information is included in the data collection template submission.

6. Further information

A summary of the key actions and timelines for practices are outlined in Appendix 2.

In the event of any disagreement, which cannot be resolved between the practice and the Medicines Optimisation team, the practice may forward an appeal describing the exceptional circumstances as to why achievement could not be attained within the specified dates described.

The Deputy Chief Pharmacist - Primary Care will constitute a panel consisting of NHS Devon and LMC representation, in equal numbers, to agree a resolution. Any appeal should be made to the Medicines Optimisation team by 31st March 2027.

7. References

1. National Institute for Health and Care Excellence (NICE). 2010. Denosumab for prevention of osteoporotic fractures in postmenopausal women (TA204). [Accessed via: <https://www.nice.org.uk/guidance/ta204> on 25th March 2026]
2. NHS England. 2023. What is a biosimilar medicine? [Accessed via: <https://www.england.nhs.uk/long-read/what-is-a-biosimilar-medicine/#prescribing-biosimilars> on 5th May 2026]
3. NHS England. 2025. Commissioning framework for best value biological medicines. [Accessed via: <https://www.england.nhs.uk/long-read/commissioning-framework-for-best-value-biological-medicines/> on 5th May 2026]
4. Medicines and Healthcare products Regulatory Agency (MHRA). 2025. Guidance on the licensing of biosimilar products. [Accessed via: <https://www.gov.uk/government/publications/guidance-on-the-licensing-of-biosimilar-products/guidance-on-the-licensing-of-biosimilar-products> on 5th May 2026]
5. NHS Devon. North and East Devon Formulary: 6.6.2 Bisphosphonates and other drugs affecting bone metabolism. [Accessed via: <https://northeast.devonformularyguidance.nhs.uk/formulary/chapters/6-endocrine/6-6-drugs-affecting-bone-metabolism/6-6-2-bisphosphonates-and-other-drugs-affecting-bone-metabolism> on 5th May 2026]
6. NHS Devon. South and West Devon Formulary: 6.6.2 Bisphosphonates and other drugs affecting bone metabolism [Accessed via: <https://southwest.devonformularyguidance.nhs.uk/formulary/chapters/6-endocrine/6-6-drugs-affecting-bone-metabolism/6-6-2-bisphosphonates-and-other-drugs-affecting-bone-metabolism> on 5th May 2026]
7. NHS Devon. Denosumab Specialised Medicines Service prescribing guideline (devon-wide). [Accessed via <https://onedevon.org.uk/download/denosumab-specialised-medicines-service-prescribing-guideline-devonwide/> on 5th May 2026]
8. PrescQipp. 2025. Hot Topic Are you ready for biosimilar denosumab (last updated 2nd February 2026) [Accessed via <https://www.prescqipp.info/our-resources/webkits/hot-topics/> on 7th May 2026]

Appendix 1 – Invoicing Information

The invoice needs to be on the surgery's headed paper.

- Please use a unique invoice number for each invoice submitted as it could be rejected if a similar number is used.
- Please state the description as 'Payment for Devon Medicines Optimisation Denosumab Scheme 2026/27' detailing the number of patients reviewed, and the numbers of patients changed for the standard payment.

Please include the following references:

- Cost centre: 316801.521611005
- Mark the invoice: XXLBAILEY to ensure correct assignment for processing.

Invoices may be submitted on Tradeshift or by mail to:

NHS Devon ICB,
QJK000 Payables N055,
PO Box 312,
Leeds,
LS11 1HP.

Alternatively, invoices can be submitted as a **non-editable pdf** via email to sbs.apinvoicing@nhs.net

Please be advised, do not include the following wording within the subject line of your email: 'statement' or 'reminder'.

Appendix 2 – Summary of key actions and timelines for practices

Key action	Timeline
Import and run clinical system search to identify patients prescribed Prolia or generic denosumab 60mg injection.	Before clinical activity commences
Review identified patients, and where clinically appropriate inform and switch eligible patients to preferred formulary brands as described in Section 2.2.	1 st July – 31 st December 2026
Submit Microsoft form detailing activity undertaken within the scheme.	By 31 st January 2027
After confirmation of the standard payment amount from the medicines optimisation team, submit an invoice as detailed in section 5.1.	By 28 th February 2027

Appendix 3 - Comparison of preferred denosumab 60mg biosimilar preparations with Prolia

Information in the following table is taken from PrescQIPP “HOT TOPICS: Are you ready for biosimilar denosumab?”

[✓ indicates information as per reference product Prolia]

Brand name	Prolia (Amgen)	Stoboclo (Celltrion)	Osvyrti (Accord)	Zadenvi (Zentiva)
Cell line	Mammalian cell line (Chinese hamster ovary cells) by recombinant DNA technology	✓	✓	✓
Licensed indications	osteoporosis in postmenopausal women and men at increased fracture risk; bone loss associated with hormone-deprivation therapy (breast/prostate cancer); glucocorticoid-induced osteoporosis.	✓	✓	✓
Administration	60 mg subcutaneous injection every 6 months. Administration should be performed by an individual who has been adequately trained in injection techniques.	✓	✓	✓
Excipients	Acetic acid, glacial; sorbitol (E420), polysorbate 20, sodium hydroxide, water for injections	Acetic acid, Sodium acetate trihydrate, Sorbitol (E420), Polysorbate 20, Water for injections	✓	✓
Storage & Shelf life	4 years in a refrigerator (2°C-8°C); Up to 30 days at room temperature (up to 25°C) stored in the outer carton in order to protect from light. It must be used within this 30 day period. Do not freeze.	✓	2 years in a refrigerator (2°C- 8°C); Up to 30 days at room temperature (up to 25°C) stored in the outer carton in order to protect from light. It must be used within this 30 day period. Do not freeze.	